NCCP 00322 · VERSION 4

Everolimus + exemestane

Both tablets are taken every day continuously. There is no planned week off. A four-week blood-test review is a monitoring interval, not a stop-and-restart treatment cycle.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Reference timing, not booked appointments. Ask who requests each test, when it is due, who reviews the result and whom to contact if it is missing. Delays can change cycle numbers; time-based monitoring remains time-based.

Check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
FBC, renal and liver profile	Baseline blood tests	Continue daily tablets as prescribed	Every four weeks or as indicated	Every four weeks or as indicated	Repeat sooner for toxicity; restart only under the agreed plan
Urea and urinary protein	Baseline urea/BUN and urine protein	Review renal symptoms/risk	Repeat renal/urine assessment as indicated	Periodic review, especially with renal risk	Investigate acute renal deterioration promptly
Cholesterol and triglycerides	Baseline lipid profile	Review baseline abnormalities	Every four weeks or as indicated	Every four weeks or as indicated	Treat abnormalities and reassess treatment tolerance
Blood glucose	Baseline glucose; assess diabetes control	Closer checks if diabetes or steroid treatment	Fasting glucose as clinically indicated	Individualised glucose monitoring	Urgent assessment of severe metabolic symptoms
Mouth care and stomatitis	Agree prevention and mouthwash plan	Watch for early soreness	Particular attention during weeks 1–8	Review symptoms at each contact	Grade and treat toxicity; separate pause/restart plan
Lung symptoms / infection	Review lung history and infection	Report new symptoms promptly	Review throughout treatment	No universal screening-CT interval	New symptoms trigger assessment now, not at the next routine scan
Bone health / DXA	Assess fracture risk and DXA need	Address identified risk factors	Individual bone-health plan	Repeat DXA according to risk and results	Reassess after fracture or treatment changes
ECG / echocardiogram	Only if clinically indicated	No regimen-mandated routine test	No regimen-mandated routine interval	Investigate symptoms/risk as appropriate	Add requirements from any new treatment
Response imaging / markers	Record measurable sites and baseline imaging	Agree first response review	Usually every 2–4 months; individualise	Adapt to disease pace; earlier for symptoms	Markers supplement, never replace, assessment

THE WHOLE TREATMENT JOURNEY

Everolimus + exemestane: the whole pathway

Postmenopausal HR-positive / HER2-negative advanced breast cancer

Both tablets are taken every day continuously. There is no planned week off. A four-week blood-test review is a monitoring interval, not a stop-and-restart treatment cycle.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Treatment NCCP	Confirm exact indication and biomarkers	Usual starting doses: everolimus 10 mg once daily and exemestane 25 mg once daily; the prescribed everolimus dose may be lower.	Both tablets are taken every day continuously. There is no planned week off. A four-week blood-test review is a monitoring interval, not a stop-and-restart treatment cycle.	Continue with safety / benefit review	Continue while the treatment is helping and side effects remain manageable, according to the consultant plan. This advanced-disease regimen has no fixed adjuvant completion date.
Surgery / radiotherapy ESMO_MBC2023	Only for an agreed local-control or symptom need	Teams decide sequence / any treatment interruption	No routine surgery or RT implied	Review any local-treatment effects	Reassess if goals or symptoms change
Safety checks NCCP · SMPC	Baseline bloods and individual risks	Early checks from the monitoring table	Regimen-specific checks continue	Review results before continuation	Resolve abnormalities / agree next plan
Scans / follow-up ESMO_MBC2023	Confirm baseline and treatment setting	Agree the next imaging milestone	Use the matching surveillance / response plan	Review benefit and symptoms	Continue setting-specific follow-up

What each check is for

1 Blood tests

Before starting: FBC, renal and liver profile, urea/BUN, urinary protein, blood glucose, cholesterol and triglycerides. Confirm the endocrine indication, review lung symptoms and infections, and check all medicines. Assess bone/fracture risk for the exemestane component. FBC, renal/liver profile, cholesterol and triglycerides every four weeks or as clinically indicated. Check fasting glucose as clinically indicated, with closer monitoring if diabetes or another medicine increases risk. Ask about mouth soreness, infection, cough/breathlessness and daily function at reviews.

Sources: NCCP · SMPC

2 Heart checks

NCCP 00322 does not mandate routine ECG or echocardiography. Cardiac tests should follow symptoms, comorbidities and other treatments, rather than a copied HER2-treatment schedule.

Sources: NCCP · SMPC

3 Imaging

Response assessment in advanced breast cancer is generally every two to four months, adapted to disease dynamics, sites, symptoms and treatment. Less frequent monitoring may suit indolent disease. New symptoms or suspected progression need earlier assessment. The team chooses the imaging method; this is not an automatic three-month CT chest/abdomen/pelvis order.

Sources: ESMO_MBC2023

4 Bone health

Exemestane can reduce bone mineral density. Review fracture risk and arrange DXA and bone protection when indicated. This regimen does not specify a single repeat-DXA interval for everyone.

Sources: NCCP · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.